



SPECIMEN

- 1 - Right iliac lymph node.
- 2 - Left ureteric resection margin.
- 3 - Bladder & prostate.

CLINICAL DETAILS

Invasive bladder cancer.

Left ureter ? is there evidence of tumour in distal part (clinically it appears so) and does this extend to proximal part.

MACROSCOPY

- 1 - Enlarged node measuring 27 x 20 x 12 mm.
- 2 - An orientated tubular structure 20 mm in length and up to 6 mm in dia. with surrounding fatty tissue.
- 3 - Specimen consists of bladder (measuring 65 x 40 x 40 mm), prostate (measuring 55 mm medio-lateral x 35 mm supero-inferiorly x 35 mm anterior-posterior) and perivesical fat. The bladder contains an ulcerated lesion measuring 25 x 20 mm involving the posterior and left lateral walls. On section the tumour appears to extend through the wall to reach the perivesical fat and reach the circumferential surgical resection margin posteriorly. No lymph nodes are identified in the perivesical fat.

- 1 - Right iliac lymph node;

FROZEN SECTION REPORT

- 1 - Frozen section report; metastatic carcinoma. Verbal report given to anaesthetics at on the - Dr and Dr

PARAFFIN SECTION REPORT

Paraffin sections confirm frozen section report of metastatic carcinoma. There is extension into extracapsular tissue.

- 2 - Left ureteric resection margin;

FROZEN SECTION REPORT

- 2 - Frozen section report; both proximal and distal ureteric resection margins are normal with no carcinoma in situ. Report to surgeon at on - Dr

PARAFFIN SECTION REPORT

Paraffin sections confirm the frozen section diagnosis of morphologically normal ureteric tissue.

- 3 - Bladder and prostate; Sections from the macroscopically identified lesion confirm a poorly differentiated transitional cell carcinoma arising from surface epithelium and extending as tumour aggregates through the bladder wall to reach the extravesical tissue. There is tumour within 1 mm of the circumferential surgical resection margin.

There is extensive necrosis within the tumour and perineural and lymphovascular invasion. There is no lymphocytic infiltrate at the tumour host interface.

Tumour is seen extending into the muscle wall of the ductus deferens.

Background urothelium is morphologically normal with no evidence of dysplasia or carcinoma-in-situ.

Sections taken from the right ureteric resection margin show small tumour foci within the soft tissue and may represent lymphovascular invasion.

The urethral resection margin of this specimen is morphologically normal with no evidence of dysplasia or carcinoma-in-situ.

The sections from the right lobe of the prostate show foci of high grade PIN.

ICD-O-3

carc
Carcinoma, urothelial NOS 8120/3
not
Carcinoma, transitional cell NOS 8120/3

Site: Bladder NOS C67.9

9/25/2/14

Sections from the left lobe are morphologically normal. There is no evidence of microacinar adenocarcinoma.

Please note the urethral resection margin and left external iliac nodes have been processed as

SUMMARY

Cystoprostatectomy and lymph node dissection; the appearances are those of a poorly differentiated (G3) transitional cell carcinoma of the bladder invading perivesical soft tissue. There is tumour within 1 mm of the circumferential surgical resection margin. No metastases are noted. G3 TCC pT3b pN2 pMx.

Pathologists -

DR

CONSULTANT

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	Date Reviewed: 1/10/14	